AM-01 Allergic Reaction / Anaphylaxis

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for acute allergic symptoms, suspected anaphylaxis, or bradykinin-mediated angioedema. Epinephrine is first-line for anaphylaxis.

Anaphylaxis recognition and immediate treatment

1. Assess	Remove a trigger when safe. Assess airway, breathing, circulation, skin, GI symptoms, and perfusion.
2. Anaphylaxis?	Respiratory compromise, hypotension/poor perfusion, or acute involvement of 2 or more body systems.
3. IM epinephrine now	1 mg/mL: 0.5 mg IM lateral thigh. A 0.3 mg autoinjector is acceptable. Repeat every 5 min while significant symptoms persist.
4. Airway + support	Position supine when tolerated. Oxygen, suction, BVM as needed. Albuterol 2.5 mg nebulized for persistent bronchospasm after epinephrine.
5. Poor perfusion	Isotonic crystalloid 500 mL IV/IO aliquots, reassessing after each; maximum 2 L. Use 250 mL aliquots if fluid-overload risk.
6. Refractory shock	Continue IM epinephrine. With Medical Control, epinephrine infusion 2-10 mcg/min; no IV epinephrine bolus under this protocol.
7. Bradykinin angioedema	Early difficult-airway plan. With Medical Control, consider TXA 1 g IV/IO over 10 min while prioritizing airway and transport.
8. Monitor + transport	Continuous ECG/SpO2/BP, 12-lead when feasible, EtCO2 when indicated, early destination notification.

Key treatment and disposition

Item	Approved action
Diphenhydramine	25-50 mg slow IV/IO or deep IM only for isolated or persistent skin symptoms with adequate BP and mental status. It never delays or replaces epinephrine.
Beta-blocked refractory shock	With Medical Control, glucagon 1-2 mg IV/IO over 5 min; prepare for vomiting and airway protection.

Every patient receiving epinephrine or showing airway, respiratory, cardiovascular, progressive, or multisystem symptoms requires transport and continuous reassessment.

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SAFETY, LINKED PATHWAYS, AND RELEASE CONTROL

Safety checks

- Anaphylaxis may occur without rash. Do not delay initial or repeat IM epinephrine for IV access, monitoring, antihistamine, corticosteroid, or transport preparation.

- Routine corticosteroids are not part of the acute anaphylaxis pathway. Albuterol treats lower-airway bronchospasm only, not laryngeal edema or shock.

- Do not give diphenhydramine to a patient with hypotension, poor perfusion, or decreased mental status.

- Do not allow a hypotensive or poorly perfused patient to stand or walk.

Linked pathways

Pathway	Use when
AR-01	Progressive airway swelling or ventilatory failure
AR-04	Persistent bronchospasm or asthma/COPD
AM-05	Persistent hypotension or another shock cause

Documentation and release control

- Document focused findings, BP/rhythm/ECG when applicable, all interventions with dose/route/time/response, serial reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.